National Cancer Control Programme (NCCP) 00579 · clinician draft

Pembrolizumab + paclitaxel + carboplatin AUC6

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: paclitaxel/carboplatin cycles 1–4

Active medicines: pembrolizumab + paclitaxel + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	National Cancer Control Programme (NCCP) NCIS V8 high-risk dedicated carboplatin AUC≥4 row Reconcile with paclitaxel premedication steroid and aprepitant interaction; issue one clear prescription, do not add overlapping courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Not a routine olanzapine-containing row.
Paclitaxel hypersensitivity premedication	Source suggested dexamethasone 20 mg orally or intravenous (IV); chlorphenamine 10 mg IV; famotidine 20 mg IV	Oral dexamethasone at 6 and 12 hours before paclitaxel, or IV 30 minutes before; H1/H2 30 minutes before With aprepitant source permits considering dexamethasone 12 mg on treatment day. Reconcile antiemetic and taxane steroids; do not stack doses. Later reduction/omission or H2 omission requires a consultant/local plan and monitoring, particularly with this 3-weekly schedule.

Additional phase-specific support

- Use the dedicated carboplatin AUC≥4 high-risk NCIS row; it has no routine olanzapine.
- Paclitaxel premedication prevents infusion reactions and must be reconciled with antiemetic dexamethasone.
- Manage muscle/joint aches according to a prescribed analgesic plan; assess neuropathy before each taxane cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00579 v5a · pNCCP00579 pages 7–8; NCIS V8 Table 1 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Maintenance: pembrolizumab

Active medicines: pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Pembrolizumab alone has minimal emetic risk: no routine continuation of the induction antiemetic course or taxane premedication.
- Prescribe symptom-directed rescue if needed and investigate unexplained nausea/vomiting for immune or other causes.
- Continue immune, renal/liver, glucose and thyroid monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00579 v5a · pNCCP 00579 treatment and supportive-care sections; NCIS V8 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required